INFORMATION TO KEEP WITH YOU

My complete treatment handbook

QUASAR modified weekly fluorouracil + folinic acid

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
7 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00428
Version 6

Folinic acid and fluorouracil injections are given once each week. The adjuvant course is 30 weeks; metastatic treatment continues only while appropriate. This weekly schedule has no planned two-week rest block.

Treatment stages

01 **Prescribed treatment course**
Adjuvant or metastatic colorectal cancer

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	9
04	Your record and contact card	15
05	Sources and further information	18

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

QUASAR modified weekly fluorouracil + folinic acid
National Cancer Control Programme (NCCP) 00428

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

Weekly fluorouracil with folinic acid can reduce recurrence risk after surgery or help control metastatic colorectal cancer. Your team confirms the intended benefit.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make deoxyribonucleic acid (the cell's genetic material) (DNA). In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

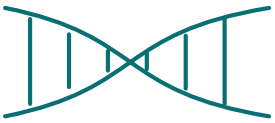
What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with DNA and RNA, the instructions cells need to grow. The same drug has different schedules; follow the prescription for this regimen.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Report diarrhoea, mouth sores, painful hands or feet and unexpected severe early symptoms. Chest pain needs urgent assessment even after a normal dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) test.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and duration

Your course

Folinic acid and fluorouracil injections are given once each week. The adjuvant course is 30 weeks; metastatic treatment continues only while appropriate. This weekly schedule has no planned two-week rest block.

Choices

Discuss the expected benefit, burdens, alternatives and how treatment fits your priorities. You can ask for more time or another explanation before deciding.

Understanding the evidence

Ask which evidence matches your treatment aim.

The QUASAR trial compared adjuvant fluorouracil/folinic acid with observation after colorectal cancer resection, mainly in stage II disease. Its group results do not predict an individual benefit or establish the metastatic indication.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment plan

QUASAR modified weekly fluorouracil + folinic acid

7-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ADJUVANT OR METASTATIC COLORECTAL CANCER

Prescribed treatment course

Start | Treatment | Review

Folinic acid and fluorouracil injections are given once each week. The adjuvant course is 30 weeks; metastatic treatment continues only while appropriate. This weekly schedule has no planned two-week rest block.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

BASELINE → TREATMENT → FOLLOW-UP

Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00428 v6 official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

QUASAR modified weekly fluorouracil + folinic acid: treatment and checks

Adjuvant or metastatic colorectal cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver NCCP 00428	Baseline tests and DPD status	Before every weekly treatment	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines NCCP 00428	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers NCCP 00428	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up

What each check is for

1 **Confirm your personal plan**

Before starting

Weekly fluorouracil with folinic acid can reduce recurrence risk after surgery or help control metastatic colorectal cancer. Your team confirms the intended benefit.

Ask / act: Agree medicines, treatment days, duration and monitoring.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before treatment

Confirm the treatment aim, duration, DPD result, blood tests, prescription and contact numbers.

STEP 2

Visits and access

Folinic acid and fluorouracil injections are given once each week. The adjuvant course is 30 weeks; metastatic treatment continues only while appropriate. This weekly schedule has no planned two-week rest block.This regimen does not require a home infusion pump.

STEP 3

If treatment is delayed

Ask for revised written treatment and support-medicine instructions. Do not make up missed doses or carry an old plan into a new phase.

STEP 4

Getting help

Keep the wallet card and medicine list available. Contact the team promptly for infection, dehydration, chest symptoms or significant toxicity.

Tests and safety

DPD before fluorouracil

Low DPD activity can cause severe toxicity. Testing helps guide the plan but cannot prevent every reaction. Unexpected severe early symptoms need urgent advice.

Blood tests

Before every weekly treatment: blood count, renal and liver profile; baseline DPD. Scans/markers follow the consultant plan.

Medicines and interactions

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment pattern	 **Days 1–7** Folinic acid and fluorouracil injections are given once each week. The adjuvant course is 30 weeks; metastatic treatment continues only while appropriate. This weekly schedule has no planned two-week rest block.
Symptoms and infection	 **Days 1–7** Report on any day; timing varies

The written plan comes first: Folinic acid and fluorouracil injections are given once each week. The adjuvant course is 30 weeks; metastatic treatment continues only while appropriate. This weekly schedule has no planned two-week rest block.

Symptoms can continue: Contact oncology for fever, sudden illness, diarrhoea, mouth soreness or inability to drink, during treatment or the break.

> **Why these bands stay broad:** Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00428 v6 official regimen ↗

> **There are no guaranteed "safe days".** Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1: Weekly treatment visit

Blood tests and symptom review before the injections. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Report symptoms before the next dose; do not wait for your next appointment if urgent.

Day 2–7: Between weekly visits

Report bowel, mouth, skin or infection symptoms promptly. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Report symptoms before the next dose; do not wait for your next appointment if urgent.

Support around the medicines

1 **Supportive medicines**

Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.

2 **Mouth, bowel and skin care**

Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.

3 **Medicine checks**

Recognise symptoms and act

Mouth / eating

Report painful ulcers or difficulty eating and drinking.

Chest / heart

New chest pain needs urgent assessment, including during an infusion.

Bowel / hydration

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

Hands / feet

Report painful redness, cracking or peeling that affects daily activity.

Blood / infection

Fever, shivers or sudden illness need immediate oncology advice.

Energy / daily life

Report fatigue that limits usual activity; ask for practical and nutrition support.

Daily life and support

Food and fluids

Aim for tolerable meals and fluids. Report weight loss or poor intake; ask for dietetic support.

Activity and rest

Adjust activity and rest to your energy. Report fatigue that limits normal tasks.

Fertility and sexual health

Ask about fertility preservation before treatment and the contraception and breastfeeding plan for all medicines. Report a possible pregnancy promptly.

Support for you and your family

Ask about counselling, psycho-oncology, practical support, transport and financial advice. Bring someone to discussions if helpful.

Questions worth asking

Which treatment plan applies to me?

Folinic acid and fluorouracil injections are given once each week. The adjuvant course is 30 weeks; metastatic treatment continues only while appropriate. This weekly schedule has no planned two-week rest block. Your team confirms any modification.

Do more side effects mean treatment is working?

No. Side effects do not reliably measure response. Report symptoms and follow the agreed review plan.

Who do I contact overnight?

Ask the team to replace example numbers with working daytime, out-of-hours and backup contact routes before leaving.

Try explaining the plan back: Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Fluorouracil treatment day

Active medicines: Folinic acid + Fluorouracil

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before the fluorouracil-only treatment visit NCCP NCIS V8 low-risk default. Review QT risk, interactions, constipation and hepatic function; clinician may select an alternative.

Medicine	Dose / route	When
Metoclopramide	10 mg orally up to three times daily as required	Rescue; prescriber confirms duration Check renal function, neurological contraindications and interactions.

Additional phase-specific support

- Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.
- Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.
- Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth / eating

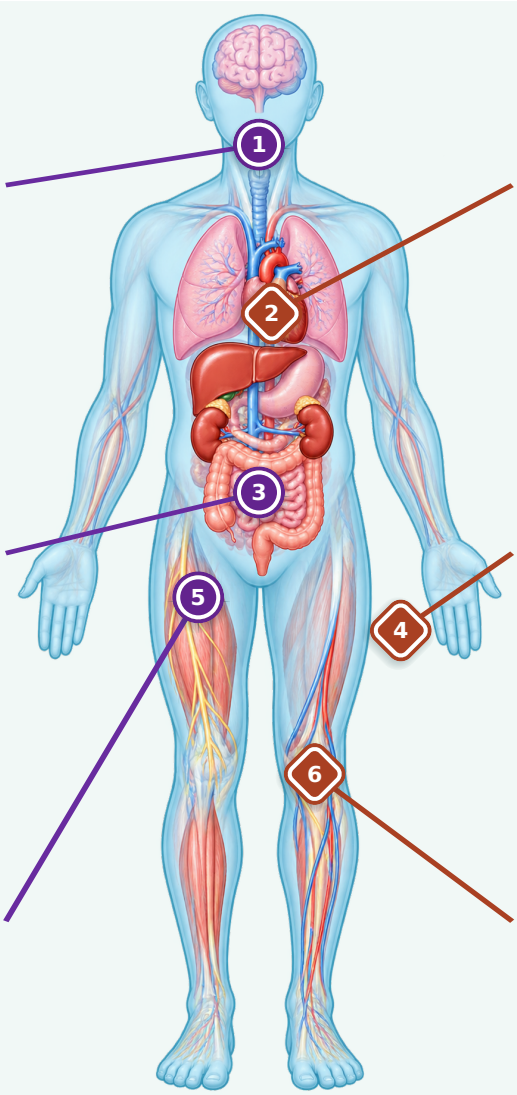
Fluorouracil

Report painful ulcers or difficulty eating and drinking.

2 Chest / heart

Fluorouracil

New chest pain needs urgent assessment, including during an infusion.



3 Bowel / hydration

Fluorouracil

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

4 Hands / feet

Fluorouracil

Report painful redness, cracking or peeling that affects daily activity.

5 Blood / infection

Chemotherapy

Fever, shivers or sudden illness need immediate oncology advice.

6 Energy / daily life

Treatment

Report fatigue that limits usual activity; ask for practical and nutrition support.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Folinic acid · Chemotherapy modulator

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999

Fluorouracil · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Hand-foot soreness	□ None	□ Redness or tingling without pain	□ Pain limits usual tasks: call	□ Pain limits washing / dressing: urgent review	□ Acute severe illness: emergency help

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Adjuvant or metastatic colorectal cancer · NCCP 00428 v6 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

QUASAR modified weekly fluorouracil + folinic acid

One treatment every week; duration as prescribed.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.32 · NCCP 00428 v6 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00428 v6 official regimen**
   https://healthservice.hse.ie/documents/6467/428_QUASAR_Modified_5FU_and_Folinic_Acid.pdf
2. **NCCP NCIS antiemetic guidance V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
3. **QUASAR primary trial report**
   https://pubmed.ncbi.nlm.nih.gov/18083404/
4. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq